

The American Health Ownership Plan (AHOP)

A market-based architecture for universal, portable, affordable American healthcare — one page

The premise. A capitalist economy runs on a healthy, mobile workforce, and the current system is engineered against that interest: prices are hidden, supply is legally cartelized, and the tax code welds coverage to employment. The diagnosis is not market failure but a market never permitted to function. AHOP is a single coherent architecture — not piecemeal reform — in which every American personally owns a catastrophic policy, a health savings account, and an income-scaled subsidy relationship.

The architecture. Layer 1: universal catastrophic coverage from competing private insurers, guaranteed issue, income-scaled deductibles (5–10% of income, capped, with a statutory risk-adjustment formula — prospective, claims-audited, reinsurance corridor — written into law, not rulemaking). A smart deductible exempts high-value care from all cost sharing: preventive services, prenatal care, and chronic-maintenance drugs, answering the RAND underinsurance critique with its own evidence. Layer 2: expanded HSAs pay for routine, shoppable care, restoring the price signal (the Singapore/Swiss existence proofs). Layer 3: a means-tested backstop for the poor and chronically ill, housed in a dedicated trust — the AHOP Trust — with statutory revenue and an adequacy floor that triggers automatic revenue adjustment: the safety net cannot be starved by inaction, only cut by a vote. Beneath all three: a load-bearing deregulation package (CON repeal, residency expansion, scope-of-practice, binding price transparency, site-neutral payments, faster generics), backed by a market-first trigger — reference-price fallback activates only in concentrated markets that miss published benchmarks, and deactivates when competition arrives.

No mandate. Guaranteed Enrollment: auto-enrollment with opt-out, continuous-coverage protection (from the GOP's own 2017 bills), and Medicare Parts B/D-style late-enrollment pricing. Mandate-level participation, zero compulsion.

Financing. Almost entirely redirection, not new taxes: phased conversion of the ~\$330B/yr employer tax exclusion — with a statutory paycheck conversion ensuring workers visibly receive the wages their premiums suppressed — plus migrated non-elderly Medicaid, CHIP, and ACA subsidy streams, and a voluntary Solidarity Share: above-cap HSA contributions remain tax-advantaged, with 25% of each above-cap dollar flowing to the Trust.

Transition. Children first (2029) on the CHIP precedent, standing up the full machinery at a tenth of the fiscal exposure; then a demographic ratchet — young adults, then adults in tranches — with early opt-in for anyone who involuntarily loses employer coverage. Medicare is grandfathered. Job loss stops being a coverage event at all: the policy and the account are personally owned, and subsidies automatically rise when income falls.

The numbers (accompanying 14-year model, adjustable scenario levers). Peak transition cost $\approx$ \$92B/yr (2031); cumulative worst point $\approx$ \$317B (2033); steady state $\approx$ \$449B/yr federal improvement by 2040 — stated honestly, much of it exclusion redirection rather than efficiency gain. The real prize is system-wide: partial deregulation success cuts national health expenditure from a 22.3% baseline to $\approx$ 20.5% of GDP by 2040 ($\approx$ \$950B/yr in economy-wide savings); full success reaches $\approx$ 18.8%. If deregulation stalls, the federal ledger barely moves — the downside case is re-plumbing at roughly zero net cost.

The pathway. Comprehensive market-based bills die in committee under the post-2017 targeted-changes doctrine — so AHOP is severable by design. Title I (deregulation) is reconciliation-compatible and can move now; Title II (children's coverage) is a self-contained bill in the most passable genre in health policy; the remaining titles are trigger-gated for the coming fiscal forcing event, most plausibly the Medicare trust-fund window of the early 2030s. Unlike every repeal-era predecessor, this framework scores as a coverage gain. It is not a bill to introduce. It is the architecture to have ready.